

HOMELESSNESS IS A HOUSING PROBLEM

A balanced research brief | Supporting evidence, counterevidence, and honest limits | October 2026

The strongest defensible claim is that housing affordability and availability help explain *differences in homelessness prevalence across communities*. Substance use, mental illness, trauma, and poverty can also independently increase *individual risk*. These findings can both be true.

PART I | Evidence for housing-market explanations

01 Colburn & Aldern (2022) | Homelessness Is a Housing Problem

Finding: Comparisons across U.S. metro areas identify rental prices and housing availability as much stronger explanations for geographic variation in homelessness than differences in substance use, mental illness, and poverty.

What it means for the housing argument: A serious answer to the claim that high homelessness counts are primarily a reflection of drug use in a city.

What we must concede: Ecological/cross-sectional comparisons do not establish what caused each individual episode, nor prove that the same coefficients apply to Chico.

[Primary source / study](#)

02 U.S. Government Accountability Office (2020)

Finding: In areas examined, a \$100 increase in median monthly rent was associated with an estimated 9% higher homelessness rate after adjustment for measured local factors.

What it means for the housing argument: An independent federal analysis directly links rental-market pressure with area homelessness rates.

What we must concede: It is an estimated association, not a controlled experiment or a guaranteed effect in every location.

[Primary source / study](#)

03 HUD Family Options Study (2010–2016)

Finding: A randomized study of 2,282 shelter-using families found that being offered a long-term housing subsidy substantially improved housing stability compared with other offers, including transitional housing and usual care.

What it means for the housing argument: This is particularly strong causal evidence that access to affordable housing can directly reduce housing instability for families.

What we must concede: Findings concern eligible families in the study, not necessarily all single adults or people with the most intensive needs.

[Primary source / study](#)

04 UCSF California Statewide Study (2023)

Finding: Approximately 3,200 adults surveyed: 90% lost last housing in California, 75% in the county of their survey. Participants described low incomes and severe housing barriers.

What it means for the housing argument: Counters a broad narrative that most unhoused Californians arrived from elsewhere and underscores local housing precarity.

What we must concede: Residency is not a direct causal test of rent and does not itself rank all pathways into homelessness.

[Primary source / study](#)

05 UCSF Behavioral Health report (2025)

Finding: Among respondents reporting regular drug use, 42% said regular use began after their first episode of homelessness.

What it means for the housing argument: Substance use does not invariably predate homelessness; timing and circumstances matter.

What we must concede: Self-reported onset does not establish that homelessness caused drug use; many report use predating homelessness.

[Primary source / study](#)

06 Butte County 2025 Point-in-Time survey

Finding: Respondents selected financial/employment concerns more often than alcohol/substance use as contributing circumstances (roughly 31% versus 17% relative to the full PIT count).

What it means for the housing argument: Useful local evidence that explanations offered by affected residents extend well beyond addiction.

What we must concede: Self-reported, overlapping responses; 17% is NOT the prevalence of substance-use disorders. PIT counts and response coverage are imperfect.

[Primary source / study](#)

PART II | Research that challenges or qualifies our argument

These studies are not dismissed. Each is weighed against the specific claim it can actually test.

07 Thompson et al. (2013) | Prospective predictors of first-time homelessness

Finding: In 30,558 previously never-homeless adults, baseline drug-use disorder predicted first-time homelessness over three years (adjusted odds ratio 2.51); alcohol-use disorder (1.34) and poverty (1.34) also independently predicted risk.

What it means for the housing argument: This is compelling evidence that addiction can be a *cause or precursor for some individuals*. It does not measure why one city has more homelessness than another or demonstrate that most episodes are caused by addiction.

What we must concede: The substance-use association remains after adjustment. We cannot honestly say addiction is merely a consequence of homelessness. Adjusted odds are not percentages or population-attributable shares.

[Primary source / study](#)

08 Johnson et al. (1997) | Social selection or social adaptation?

Finding: Among 303 people homeless or at risk in Cook County, drug abuse was associated with a first homeless episode; earlier homelessness also predicted later onset of alcohol and drug abuse symptoms.

What it means for the housing argument: Findings support a bidirectional relationship rather than a simple addiction-only explanation.

What we must concede: Drug abuse can precede homelessness and contribute to it; the 303-person local sample and age of the study limit broad generalization.

[Primary source / study](#)

09 Childhood maltreatment and psychiatric symptoms (2024)

Finding: A longitudinal cohort found 25.6% with documented childhood maltreatment had ever experienced homelessness by the first adult interview versus 12.3% of controls; psychiatric symptoms predicted future episodes.

What it means for the housing argument: Demonstrates that trauma and mental health matter to individual risk; does not rebut the role of housing affordability in area-level prevalence.

What we must concede: A housing-only account of every personal pathway would ignore substantial evidence of trauma and psychiatric vulnerabilities.

[Primary source / study](#)

10 McVicar, Moschion & van Ours (2015) | Australia

Finding: In a two-year panel of 1,325 at-risk Australians, much of the substance-use/homelessness association weakened after accounting for individual characteristics; risky alcohol use was an important possible exception.

What it means for the housing argument: Challenges simplistic one-way causation and shows correlated vulnerabilities can be mistaken for direct effects.

What we must concede: Risky drinking may increase homelessness risk, and Australian results cannot simply be assumed to describe Chico.

[Primary source / study](#)

11 National Academies (2018) | Supportive housing and health

Finding: Randomized trials reviewed found supportive housing improved housing stability without consistent significant improvements in substance use or psychiatric symptoms relative to controls.

What it means for the housing argument: Housing can succeed as housing even when health needs remain; supportive services should remain available.

What we must concede: Housing alone is not demonstrated to cure addiction or mental illness; comparisons often included treatment access for controls as well.

[Primary source / study](#)

PART III | Where the evidence does and does not take us

Supported: Rental costs and supply matter greatly to aggregate homelessness; stable affordable housing improves housing outcomes in tested populations; addiction and psychiatric conditions increase risk for some individuals; pathways can run in both directions.

Not established: That housing costs explain every episode; that drug use is never causal; that addiction is the dominant population-level cause across cities; that providing housing by itself resolves every medical or behavioral-health need.

Why both kinds of research matter: Comparing cities asks why homelessness is more prevalent in one housing market than another. Following individuals asks which experiences raise a particular person's risk. Neither answers the other question by itself.

A defensible public statement

Homelessness is fundamentally a housing problem at the community level, but people lose housing through many interacting pathways. Strong research connects high rents and scarce affordable units to higher homelessness rates. Longitudinal research also shows that addiction, mental illness, and trauma can increase individual vulnerability. Both realities call for more affordable homes *and* accessible, voluntary treatment and supports, without criminalizing homelessness or requiring everyone to follow the same path.

How to apply this to Chico

Compare local rents, vacancy rates, housing production by affordability level, and incomes over time. Pair these indicators with CoC counts, inflows into homelessness, shelter exits to permanent housing, and returns to homelessness. Report survey limitations and keep separate the share reporting substance use as a contributing cause and the prevalence of substance-use conditions.

This brief evaluates published research, not individual motivations or evidence of coordinated profiteering. Causation, policy effectiveness, and financial incentives require different kinds of proof.

PART IV | THE GEOGRAPHIC TEST

Why do communities with similar substance-use challenges have different homelessness rates?

If drug use is the main explanation for differences in homelessness between communities, why do places with comparable substance-use problems sometimes have very different homelessness rates?

What the research says. Colburn and Aldern (2022) compared U.S. metropolitan areas and found that housing-market characteristics, including rents and vacancy, were more closely associated with differences in homelessness rates than measures such as substance use and mental illness. The GAO (2020) independently identified a significant association between local rent levels and homelessness. These are population-level findings, not judgments about individual people.

Why this challenges an addiction-first explanation. If substance use alone were the dominant explanation for where homelessness is highest, a comparable pattern of homelessness across places with similar substance-use prevalence would be expected, other factors being equal. Instead, researchers find that affordability and housing availability contribute substantially to geographic variation. Housing markets can determine how easily an individual crisis becomes the loss of a home.

What the argument does NOT prove. It does not establish that two specific cities have identical substance-use rates, that rent is the sole cause of homelessness, or that all studies measure substance use in comparable ways. Local treatment access, income, climate, household structure, prevention efforts, and measurement differences may matter. Geographic correlation is not by itself conclusive causal proof.

How opposing studies fit. Thompson et al. (2013) found that pre-existing drug-use disorders predicted greater individual odds of first-time homelessness. That result should be acknowledged. It is compatible with housing-market research: addiction can raise an individual's vulnerability while costly, scarce housing helps explain why more vulnerable people become homeless in one community than another.

A testable question for Chico. Compare Chico/Butte County with carefully selected similar communities using the same years and definitions: rents relative to income, vacancy rates, affordable units, homelessness counts, substance-use prevalence, and access to housing assistance. Do not claim a matched comparison has already been proved without doing that analysis.

Public-facing takeaway

Addiction can increase a person's risk of becoming homeless. But why do communities facing similar personal hardships have such different homelessness rates? The housing market is an essential part of the answer.

PRIMARY SOURCES

Colburn & Aldern (2022), *Homelessness Is a Housing Problem*

GAO (2020), *Homelessness: Better HUD Oversight of Data Collection Could Improve Estimates*

Thompson et al. (2013), *Prospective predictors of first-time homelessness*